

SURGICAL PATHOLOGY REPORT

* Final Report *

ICD-O-3
Carcinoma, urothelial,
papillary 8130/3
Site: Bladder, lateral wall
C67.2
9/23/26/14

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MICROSCOPIC EXAMINATION:

A-C: See final microscopic diagnosis.

D: Sections of the bladder representing the section of the polypoid mass in the left lateral wall shows a moderate to poorly differentiated papillary urothelial carcinoma (D4-9), grade 3 of 4, extending into the superficial part of the muscularis propria. Associated with invasive tumor are foci of in situ urothelial carcinoma (flat lesion) which mainly involve the left lateral and posterior wall of the bladder. The left bladder wall also shows chronic inflammation, fibrosis and changes consistent with previous biopsy site. The rest of the bladder mucosa shows extensive chronic inflammation with cystitis glandularis and cystitis cystica. All resection margins of the bladder are free of dysplasia and malignancy. The prostatic urethra show polypoid cystitis (D23). Sections of the prostate show marked chronic inflammation; otherwise, the prostate is free of dysplasia or malignancy.

INTRAOPERATIVE FROZEN CONSULTATION:

AFS: Right distal ureter:

- benign

BFS: Left distal ureter:

- benign

CFS: Apical urethral margin:

- prostatitis with calcification

SFS: Sigmoid polyp:

- thrombosis, granulation tissue and tattoo

GROSS EXAMINATION:

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A: The specimen is received fresh from the O.R. and labeled "Rt. distal ureter-f/s". It consists of a segment of ureter with surrounding fat measuring 0.5 x 0.4 x 0.3 cm. Entirely submitted for frozen section in cassette AFS.

B: The specimen is received fresh from the O.R. and labeled "Lt. distal ureter-f/s". It consists of a segment of ureter with surrounding fat measuring 0.4 x 0.3 x 0.3 cm. The ureter itself measures 0.3 x 0.3 cm. Entirely submitted for frozen section in cassette BFS.

C: The specimen is received fresh from the O.R. and labeled "Apical urethral margin-f/s". It consists of two pink-tan soft tissue fragments measuring 2 and 0.7 cm in greatest diameter. The specimen is entirely submitted for frozen section in cassette CFS.

D: The specimen is received fresh from the O.R. and labeled "Bladder/prostate". It consists of radical cystoprostatectomy specimen measuring 0.8 x 15 x 5 cm overall. The peritoneum measures 23 x 13 by less than 0.1 cm. It is pink-tan, smooth, glistening and unremarkable. The prostate and lower bladder are entirely inked black. Red ink is superimposed at the right side of the prostate. The prostate itself measures 4.5 x 4 x 3.5 cm. Opening the prostate anteriorly reveals a prostatic urethra to measure 3.5 x 1 cm without excavation. The verumontanum measures 0.7 x 0.3 x 0.2 cm. It is unremarkable. The bladder measures 10 x 7 x 4 cm. Opening the bladder anteriorly shows a pink-tan polypoid mass on the upper part of the left anterior wall which measures 2.5 x 2.5 x 1.5 cm. The polypoid mass is surrounded by a velvety hemorrhagic area measuring 1.2 x 1.7 cm which is surrounded by white-tan scar measuring 1.5 cm. The mucosa in the posterior bladder wall is granular and edematous. The rest of the bladder mucosa is pink-tan and unremarkable. Sectioning the polypoid mass reveals that the tumor extend grossly to the superficial muscle but does not extend to the deep muscle or the surrounding fat. The attached right ureter measures 6 cm in length and 0.6 cm in circumference with normal appearing mucosa. The attached left distal ureter measures 5 cm in length by 0.5 cm in circumference with normal appearing mucosa. The right seminal vesicle measures 2.5 x 2 x 0.5 cm, and the left seminal vesicle measures 2.5 x 1.7 x 0.5 cm. Both the right and left seminal vesicles are grossly unremarkable. The right vas deferens measures 4 cm in length by 0.2 cm in diameter, and the left vas deferens measures 4.5 cm in length by 0.2 cm in diameter, and are unremarkable. No perivesical lymph nodes are identified. Representative sections are submitted in 30 cassettes.

E: The specimen is received in formalin and labeled "Lt. para aortic LN". It consists of multiple fragments of yellow fatty tissue with lymph nodes, measuring 3 x 2 x 0.5 cm. The specimen is entirely submitted in three cassettes.

F: The specimen is received in formalin and labeled "Lt. common iliac LN". It consists of multiple fragments of yellow fatty tissue with lymph nodes, measuring 6.5 x 3.8 x 2.1 cm in aggregate. Entirely submitted in six cassettes.

G: The specimen is received in formalin and labeled "Rt. para caval LN". It consists of a 1.5 x 1 x 0.5 cm yellow fatty tissue fragment. Serially sectioned and totally submitted in one cassette.

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H: The specimen is received in formalin and labeled "Rt. common iliac LN". It consists of multiple fragments of yellow fatty tissue with lymph nodes measuring 5.5 x 3.5 x 2 cm in aggregate. The specimen is entirely submitted in six cassettes.

J: The specimen is received in formalin and labeled "Rt. LN of cloquet". It consists of a 1.5 x 1 x 0.5 cm aggregate of yellow fatty tissue. Sectioning reveals one lymph node. The tissue is totally submitted in one cassette.

K: The specimen is received in formalin and labeled "Rt. external iliac LN". It consists of multiple fragments of yellow fatty tissue with lymph nodes, measuring 4.5 x 4 x 2.5 cm. The specimen is entirely submitted in six cassettes.

L: The specimen is received in formalin and labeled "Rt. obturator/hypogastric LN". It consists of multiple fragments of yellow fatty tissue with lymph nodes, measuring 5.5 x 5 x 2 cm in aggregate. The specimen is entirely submitted in five cassettes.

M: The specimen is received in formalin and labeled "Lt. LN of cloquet". It consists of 1.5 x 1 x 0.5 cm yellow fatty tissue fragment. Sectioning reveals one lymph node. Lymph node and tissue are totally submitted in one cassette.

N: The specimen is received in formalin and labeled "Lt. external iliac". It consists of multiple fragments of yellow fatty tissue with lymph nodes measuring 2.5 x 3 x 2 cm in aggregate. Entirely submitted in four cassettes.

O: The specimen is received in formalin and labeled "Lt. obturator/hypogastric L.nodes". It consists of multiple fragments of yellow fatty tissue containing multiple lymph nodes measuring in aggregate 2.5 x 3 x 2.5 cm. Entirely submitted in six cassettes.

P: The specimen is received in formalin and labeled "Pre sacral LN". It consists of multiple fragments of yellow fatty tissue containing multiple lymph nodes measuring in aggregate 2.5 x 2 x 1.5 cm. The specimen is entirely submitted in three cassettes.

Q: The specimen is received in formalin and labeled "Lt. proximal ureter". It consists of a fragment of ureter measuring 0.5 x 0.2 cm. The specimen is bisected and totally submitted in one cassette.

R: The specimen is received in formalin and labeled "Rt. proximal ureter". It consists of a fragment of ureter measuring 0.4 x 0.2 cm. The specimen is bisected and entirely submitted in one cassette.

S: The specimen is received fresh from the O.R. and labeled "Sigmoid polyp-f/s". It consists of a 1.4 x 0.7 x 0.3 cm pink-tan mucosal tissue fragment. In one margin, there is inked area designated as tattoo. The specimen is bisected and one-half is submitted for frozen section and the other half for permanent section in one cassette.

SECTIONS:

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AFS: frozen section, right distal ureter
BFS: frozen section, left distal ureter
CFS: frozen section, apical urethral margin
D1: bladder/prostate; representative section from anterior bladder wall
D2: representative section from right lateral bladder wall
D3: right ureterovesical junction
D4: dome of bladder
D5: polypoid mass from left lateral wall
D6,7: contiguous sections of polypoid mass with deep muscle and fat
D8,9: polypoid tumor
D10-12: hemorrhagic area on the scar tissue
D13: representative sections from posterior bladder wall
D14: left ureterovesical junction
D15: trigone
D16: right apex, prostate
D17: left apex, prostate
D18: right distal prostate
D19: left anterior distal prostate
D20: left posterior distal prostate
D21: right anterior mid prostate
D22: right posterior mid prostate
D23: left anterior mid prostate
D24: left posterior mid prostate
D25: right anterior proximal prostate
D26: right posterior proximal prostate/seminal vesicle junction
D27: left anterior proximal prostate
D28: left posterior proximal prostate/seminal vesicle junction
D29: right seminal vesicle and right vas deferens
D30: left seminal vesicle and left vas deferens
E1-3: left para-aortic lymph node - all embedded
F1-6: left common iliac - all embedded
G: right paracaval lymph node - all embedded
H1-6: right common iliac lymph node - all embedded
J: right lymph node of Cloquet - all embedded
K1-6: right external iliac lymph node - all embedded
L1-5: right obturator/hypogastric lymph node - all embedded
M: left lymph node of Cloquet - all embedded
N1-4: left external iliac - all embedded
O1-6: left obturator/hypogastric lymph nodes - all embedded
P1-3: presacral lymph node - all embedded
Q: left proximal ureter

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R: right proximal ureter
SFS: frozen section, sigmoid polyp (including deeper sections)

Signature Line

CLINICAL INFORMATION:

Pre-Op Dx: Bladder cancer

Post-Op Dx: Same as pre-op

SPECIMEN SOURCE:

AFS: "Rt. distal ureter-f/s"
BFS: "Lt. distal ureter-f/s"
CFS: "Apical urethral margin-f/s"
D: "Bladder/prostate"
E: "Lt. para aortic LN"
F: "Lt. common iliac LN"
G: "Rt. para caval LN"
H: "Rt. common iliac LN"
J: "Rt. LN of cloquet"
K: "Rt. external iliac LN"
L: "Rt. obturator/hypogastric LN"
M: "Lt. LN of cloquet"
N: "Lt. external iliac"
O: "Lt. obturator/hypogastric L.nodes"
P: "Pre sacral LN"
Q: "Lt. proximal ureter"
R: "Rt. proximal ureter"
SFS: "Sigmoid polyp-f/s"

COMMENT:

Permanent sections of the colonic wall from the site of previous sigmoid polypectomy reveal an area of adenomatous mucosa within the granulation tissue and focally on the surface. I cannot exclude the possibility that the adenomatous mucosa in the granulation tissue is a focus of residual carcinoma. The change is not seen on review of the frozen sections and only focally on the permanent sections of this specimen. That includes deeper sections of the paraffin block. The surface adenomatous change is close to an inked margin but on deepers is not seen to involve it.

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DIAGNOSIS:

RADICAL CYSTOPROSTATECTOMY WITH RADICAL LYMPH NODE DISSECTION:

RIGHT DISTAL URETER (A):

FROZEN SECTION DIAGNOSIS:

- NO CARCINOMA IN SITU

FINAL DIAGNOSIS:

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LEFT DISTAL URETER (B):

FROZEN SECTION DIAGNOSIS:

- NO CARCINOMA IN SITU

FINAL DIAGNOSIS:

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

APICAL URETHRAL MARGIN (C):

FROZEN SECTION DIAGNOSIS:

- PROSTATITIS WITH CALCIFICATION

- NO CARCINOMA IN SITU

FINAL DIAGNOSIS:

- BENIGN PROSTATIC TISSUE WITH CHRONIC INFLAMMATION AND CALCIFICATION

- NO DYSPLASIA OR MALIGNANCY IDENTIFIED

BLADDER/PROSTATE (D):

BLADDER:

- INVASIVE POORLY DIFFERENTIATED PAPILLARY UROTHELIAL CARCINOMA, GRADE3/4

- VASCULAR SPACE INVASION BY TUMOR

- THE TUMOR INVADES THE SUPERFICIAL PORTION OF THE MUSCULARIS PROPRIA

- UROTHELIAL CARCINOMA IN SITU (FLAT LESION), INVOLVING LEFT LATERAL WALL AND POSTERIOR BLADDER WALL

- CHRONIC INFLAMMATION FIBROSIS AND FOREIGN BODY GIANT CELL REACTION

- CONSISTENT WITH PREVIOUS BIOPSY INVOLVING LEFT POSTERIOR BLADDER WALL

- CHRONIC CYSTITIS, CYSTITIS GLANDULARIS AND CYSTITIS CYSTICA INVOLVING MOST OF THE BLADDER MUCOSA

- RESECTION MARGINS, FREE OF MALIGNANCY

PROSTATE:

- BENIGN ADENOMATOUS HYPERPLASIA AND CHRONIC PROSTATITIS WITH CALCIFICATION

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- PROSTATIC URETHRA SHOWING POLYPOID URETHRITIS WITH REACTIVE CHANGES
- NO PROSTATIC ADENOCARCINOMA OR UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED
- BILATERAL SEMINAL VESICLES, NO SIGNIFICANT HISTOPATHOLOGIC CHANGES

LEFT PARA AORTIC LYMPH NODES (E):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

LEFT COMMON ILIAC LYMPH NODES (F):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

RIGHT PARA CAVAL LYMPH NODES (G):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

RIGHT COMMON ILIAC LYMPH NODES (H):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

RIGHT LYMPH NODE OF CLOQUET (J):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODES EXAMINED (0/1)

RIGHT EXTERNAL ILIAC LYMPH NODES (K):

- NO MALIGNANCY IDENTIFIED IN NINE LYMPH NODES EXAMINED (0/9)

RIGHT OBTURATOR/HYPOGASTRIC LYMPH NODES (L):

- NO MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

LEFT LYMPH NODE OF CLOQUET (M):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODES EXAMINED (0/1)

LEFT EXTERNAL ILIAC (N):

- NO MALIGNANCY IDENTIFIED IN 13 LYMPH NODES EXAMINED (0/13)

LEFT OBTURATOR/HYPOGASTRIC LYMPH NODES (O):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

PRE SACRAL LYMPH NODE (P):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

LEFT PROXIMAL URETER (Q):

- BENIGN URETER
- NO DYSPLASIA OR MALIGNANCY IDENTIFIED

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RIGHT PROXIMAL URETER (R):

- BENIGN URETER
- NO DYSPLASIA OR MALIGNANCY IDENTIFIED
- FRAGMENT OF BENIGN SMALL BOWEL MUCOSA IS IDENTIFIED ACCOMPANYING THE URETER

SIGMOID POLYP (S):

FROZEN SECTION DIAGNOSIS:

- THROMBOSIS, GRANULATION TISSUE AND TATTOO

FINAL DIAGNOSIS:

- BENIGN COLON MUCOSA WITH GRANULATION TISSUE AND A TATTOO
- FOCAL COLONIC MUCOSA WITH ADENOMATOUS CHANGE (SEE COMMENT)

LYMPH NODE SUMMARY: NO MALIGNANCY IDENTIFIED IN A TOTAL OF 58 LYMPH NODES EXAMINED (0/58)

PATHOLOGIC TNM STAGE: BLADDER: pTis, T2bN0MX

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Electronically signed by
Verified:

Completed Action List:

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Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History	✓	✓
Dual/Synchronous Primary Noted		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	gpc	Date Reviewed: 1/16/2014